MedAgent Clinical Decision Support

Case ID: case_dc5975f6a3 | **Status:** EDIT | **Physician:** Dr. Alex Vance | **Date:** 2026-10-13 18:06 UTC

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Patient Intake & Presentation

Patient: Age: 72, Sex: female
Notes: 72yo female with type 2 diabetes and hypertension presenting with progressive lethargy, nausea, oliguria for 24 hours. Currently taking lisinopril 20mg and spironolactone 25mg daily. Denies chest pain, denies hemoptysis, denies dyspnea.

Clinical Summary / Impression

Patient clinical evaluation demonstrates features consistent with Acute Kidney Injury. Diagnostic synthesis is supported by key multimodal findings including oliguria, Renal Impairment, Hyperkalaemia. Differential considerations include Severe Hyperkalemia, Metabolic Acidosis. Targeted diagnostic workup and serial clinical monitoring are recommended to confirm etiology and guide definitive management.

Ranked Differential Diagnoses

#	Condition	Likelihood	Key Supporting Findings	Citations
1	Acute Kidney Injury	High	oliguria, Renal Impairment, Hyperkalaemia, Metabolic Acidosis	S_AcuteK_1, S_Acu
2	Severe Hyperkalemia	Moderate	Hyperkalaemia, Renal Impairment, Metabolic Acidosis	S2, S3
3	Metabolic Acidosis	Moderate	Metabolic Acidosis, Renal Impairment	S6, S_Chroni_1
4	Hyponatremia	Moderate	lethargy, Hyponatraemia	S1
5	Chronic Kidney Disease	Moderate	Renal Impairment, Hyperkalaemia	S_Chroni_1, S_Chro

Laboratory Findings & Identified Patterns

Identified Clinical Patterns: Renal Impairment (urgent); **Hyperkalaemia** (urgent); **Hyponatraemia** (soon); **Metabolic Acidosis** (soon)

Physician Attestation: I, Dr. Dr. Alex Vance, hereby confirm that I have reviewed, verified, and adjusted this decision-support synthesis. Decision: **EDIT**.
Reviewer Notes: No specific notes recorded.
Signature: _____ **Date:** 2026-10-13